

Title

Rapid Rate Control in Acute Decompensated Heart Failure With Rapid Atrial Fibrillation: Benefits of Intensive Beta-Blockade

Abstract

Achieving prompt ventricular rate control in the setting of acute heart failure and atrial fibrillation may accelerate hemodynamic recovery and symptom resolution. Forty patients with acute decompensated heart failure, reduced ejection fraction, and uncontrolled atrial fibrillation (heart rate >130) were randomly assigned to intensive rate control using combination beta-blockade or to standard-care rate control. The intensive-control group achieved target heart rate significantly faster, with median time to goal approximately 1.5 hours versus 4 hours in the standard group. At the 2-hour mark, intensive-control participants demonstrated greater absolute heart rate reduction (approximately 40% decrease versus 23% in standard care). Clinical endpoints including dyspnea improvement and resolution of pulmonary congestion were more rapidly attained in the intensive-control arm. Blood pressure remained stable in both groups without excessive hypotensive episodes. At 24 hours, hemodynamic parameters converged between groups. Intensive rate-control strategies offer early symptomatic benefit in acute heart failure complicated by rapid atrial fibrillation.